cent were put on treatment. The two new drugs for DR-TB—bedaquiline and delamanid, whose patents are set to expire this year—are not on the market and are provided free of cost by the government as of now. Price is a barrier to mainstreaming them: on the market, they would have cost roughly Rs 2,000 a day, says Dr Nayar. The government, however, is also pushing to develop newer shorter-duration drugs for DR-TB. A new drug by ICMR is being tested in Mumbai, which could reduce treatment to six months.

Many shorter-duration, more effective drugs are now available globally. There are also oral alternatives to injectibles, and drugs with fewer side-effects. These must be brought to India and compulsory licensing undertaken to lower costs. "How many people will approach the courts to ensure they receive the correct drugs for their resistant TB? The time and cost of legal action keep them away. The government must ensure there is no monopoly over these essential drugs due to patents. If we don't get generic versions for bedaquiline and delamanid, it will be an opportunity lost," says advocate Anand Grover, who has helped TB patients receive drugs through court orders in the past. Not all, however, plump for decentralisation. AIIMS pathologist Dr Purva Mathur, for instance. "Bacteria are intelligent organisms that can adapt quickly.... That is why they are older than humans. They're getting used to the drugs we have, and we aren't developing enough new drugs," she says. A wider arsenal, in that view, may be a more logical objective than the popularisation of just two drugs.

Since affordability remains a bottleneck, the government has launched several schemes to help patients. The Jan Arogya Yojana is one lifeline, the government also provides financial assistance of Rs 500 per patient under the Ni-kshay Poshan Yojana and free medicines through government hospitals. Around 7.1 million TB patients have got assistance worth around Rs 2,100 crore since 2018 under it. The government is also hoping that cheaper equivalents of bedaquiline and delamanid will further lower the treatment cost in late 2023; both drugs comprise 35-70 per cent of the cost of the DR-TB treatment currently. The government is also providing nutrition kits under various schemes, since poor nutrition is a leading cause of TB. A 2022 study in PubMed by researchers at Mangalore's Yenepoya University estimated that undernutrition contributed to 902,000 of India's TB cases. According to WHO, people with undernutrition are three times more at risk of developing TB. Per its data, globally in 2021, some 2.2 million new TB cases were attributable to undernutrition alone, including 655,000 in India.

As part of the government's latest TB-Mukt Panchayat Initiative, the support of 250,000 gram panchayats will be enlisted to raise awareness about TB, eliminate stigma and monitor service delivery. The panchayats can also help identify TB survivors and involve them in their communication and counselling strategies. India also has a system in place to monitor the progress of TB elimination in each district. A new TB Preventive Therapy has been launched too, for those who are at high risk of developing the disease. As part of the new regime, the first-line drugs have to be taken only once a week for 12 weeks, instead of one drug daily for six months.



"INDIA'S LEADERSHIP IS TAKING THE ELIMINATION OF TB SERIOUSLY. THIS IS AN EMBLEMATIC DECISION, AS IT COMES FROM **A COUNTRY THAT IS MOST AFFECTED** BY TB"

–LEENA MENGHANEY,
Regional Head, MSF Access Campaign, South Asia

INDIA'S WAY

India's efforts are being lauded by global stakeholders. "India's leadership," says Leena Menghaney, regional head of the MSF Access Campaign, South Asia, "is taking the elimination of TB seriously. This decision is such an emblematic one, coming from a country that is one of the most affected by TB. The generic competition and the scale-up of the shorter oral regimen containing bedaquiline will have a price-lowering effect." Dr Lucica Ditiu, executive director of the Stop TB Partnership, echoed the same at the Varanasi summit. "India is inspiring the world in TB elimination. Every country should have the Ni-kshay Mitra initiative. I salute the way it is happening at sub-national levels too, not in pilot mode but at scale," she said. The Global Fund for Malaria, TB and HIV is setting aside over 50 per cent of the $500 million allocation to India to TB in its forthcoming 2024-27 grant cycle, which is expected to improve treatment coverage, access and affordability further.

Not everyone is quite sure that India will be able to eliminate TB by 2025. "I think most people know the deadline is unrealistic," says Blessina Kumar. "The government is setting many initiatives in motion, but they aren't being seen all the way through." Mehra shares her doubts. "No, it is not possible," he says, about the deadline. "Even if we were on the way, there has been a setback due to Covid. The government's current framework is limited and doesn't factor in ground realities." But the question isn't whether we can end TB by 2025. The goal is strictly aspirational; what it can do is accelerate the momentum on the intent to eliminate a disease that has consumed us for so long. ■

(With inputs from Himanshu Shekhar)